

METFORMIN REVA

Film Coated Tablets



Composition:

- Each METFORMIN REVA 500 Film Coated Tablet contains: metformin hydrochloride 500 mg.
- Each METFORMIN REVA 850 Film Coated Tablet contains: metformin hydrochloride 850 mg.
- Each METFORMIN REVA 1000 Film Coated Tablet contains: metformin hydrochloride 1000 mg.

Mechanism of Action:

Metformin is a biguanide with antihyperglycaemic effects, lowering both basal and post-prandial plasma glucose. It works by decreasing hepatic glucose production, decreasing intestinal absorption of glucose, and improving insulin sensitivity.

Pharmacokinetics:

After an oral dose of metformin tablet, maximum plasma concentration (C_{max}) is reached in 2.5 hours (t_{max}). metformin absorption is saturable and incomplete. Plasma protein binding is negligible. Metformin is excreted unchanged in the urine. The elimination half-life is approximately 6.5 hours.

Indications:

METFORMIN REVA is indicated for the treatment of type 2 diabetes mellitus, particularly in overweight patients. METFORMIN REVA may be used as monotherapy or in combination with other oral anti-diabetic agents, or with insulin.

Contraindications:

- Hypersensitivity to metformin or to any of the excipients.
- Any type of acute metabolic acidosis (such as lactic acidosis, diabetic ketoacidosis).
- Diabetic pre-coma.
- Severe renal failure (GFR <30 mL/min).
- Acute conditions with the potential to alter renal function such as: dehydration and shock.
- Disease which may cause tissue hypoxia such as: decompensated heart failure, respiratory failure.
- Hepatic insufficiency, acute alcohol intoxication, alcoholism.

Warnings & Precautions:

- Rare lactic acidosis may occur due to metformin accumulation.
- May cause ovulation in anovulatory and premenopausal PCOS patients.
- In patients with stable chronic heart failure, metformin may be used with a regular monitoring of cardiac and renal function.
- Increased risk of severe hypoglycemia in some cases such as elderly and dehydration have been reported.

Pregnancy & Lactations:

Pregnancy: Pregnancy category B. During pregnancy, it is recommended that diabetes is not treated with metformin, but insulin be used to maintain blood glucose levels as close to normal as possible.

Lactation: Metformin is excreted into breast milk; breast-feeding is not recommended during metformin treatment.

Driving & Using Machines:

Metformin monotherapy has no effect on the ability to drive or to use machines.

Drug Interactions:

- Alcohol: Alcohol intoxication is associated with an increased risk of lactic acidosis.
- Corticosteroids, beta2-stimulators and diuretics, owing to hypoglycemic activity: More frequent blood glucose monitoring may be required. If necessary, adjust the metformin dosage.

- Inhibitors of OCT1 (such as verapamil) may reduce efficacy of metformin.

- Inducers of OCT1 (such as rifampicin) may increase absorption and efficacy of metformin.

- Inhibitors of OCT2 (such as cimetidine, trimethoprim, isavuconazole) may decrease the renal elimination of metformin and thus lead to an increase in metformin plasma concentration.

Side Effects:

Common: Nausea, vomiting, diarrhoea, abdominal pain, loss of appetite and taste disturbance.

Very rare: Lactic acidosis, decrease of vitamin B12 absorption, erythema, urticaria and hepatitis.

Dosage & Administration:

Adults: The usual starting dose is 500 mg or 850 mg 2 or 3 times daily given during or after meals. After 10 to 15 days the dose should be adjusted based on blood glucose measurements.

Combination with insulin: The usual starting dose of 500 mg or 850 mg 2 or 3 times daily, while insulin dosage is adjusted based on blood glucose measurements.

The maximum recommended dose of METFORMIN REVA is 3 g daily, taken as 3 divided doses.

Children from 10 years of age and above: The usual starting dose is one tablet of 500 mg or 850 mg metformin once daily, given during or after meals. After 10 to 15 days the dose should be adjusted based on blood glucose measurements. The maximum dose is 2 g daily, taken as 2 or 3 divided doses.

Renal impairment: Depending on the glomerular clearance rate, the dose is as follows:

GFR (mL/min)	Total maximum daily dose (divided into 2-3 daily doses)	Additional considerations
60-89	3000 mg	Dose reduction may be considered in relation to declining renal function
45-59	2000 mg	The starting dose is at most half of the maximum dose. The risk of lactic acidosis must be considered.
30-44	1000 mg	
<30	-	Metformin is contraindicated

Overdose:

High overdose of metformin or concomitant risks may lead to lactic acidosis. Lactic acidosis is a medical emergency and must be treated in hospital. The most effective method to remove lactate and metformin is haemodialysis.

Storage:

Keep out of reach of children. Store below 25 °C.

Packaging:

Each METFORMIN REVA (500-850-1000) carton box contains 20 Or 30 Film Coated Tablets in two or three blister strips.

- A medication is a product which affects your health, and it's consumption contrary to instructions is dangerous for you.
- Follow strictly the doctor's prescription, the method of use and the instructions of the pharmacist who sold the medication.
- The doctor and the pharmacist (experts in medicine, it's benefits and risks, do not repeat the same prescription without consulting your doctor.
KEEP THE MEDICATIONS OUT OF REACH OF CHILDREN
Council of Arab Health Ministers, Union of Arab Pharmacies

Reva Pharmaceutical Industry - Syria

